



CONFIDENTIAL PITCH DECK

Varada Nexus Private Limited

The operating system for asset-heavy execution. A doctor-led company building a proprietary Enterprise Management System (EMS) that runs hospitals and asset-heavy projects end to end — and scales as SaaS.

01 Executive summary

Entity	Varada Nexus Private Limited (Pvt Ltd)
CIN / PAN	U43121AP2025PTC117741 · AAKCV7495B
Incorporated	12 Feb 2025 · Andhra Pradesh
Classification	Healthcare & Lifesciences · Healthcare IT · Platform
Stage	Validation · Pre-revenue · EMS live internally
Team	10 people · 2 directors · doctor-led

Why we win

Execution in hospitals and asset-heavy industries is fragmented across vendors, chat groups and spreadsheets. We replace it with one doctor-led operating system — already running our own projects — and turn that know-how into licensable software.

Proprietary EMS

Nine verticals

SaaS roadmap

Recurring revenue

- Live proprietary EMS already running our own projects
- Nine verticals coordinated on one horizontal platform
- Pre-revenue today, with a clear path to recurring SaaS

02 The problem

Asset-heavy execution is fragmented, opaque and hard to control

Hospital promoters and operators coordinate **8+ workstreams** — planning, approvals, procurement, staffing, logistics, documentation, billing and finance — across disconnected tools. There is no single source of truth, and no repeatable playbook.

The pain compounds for first-time and regional promoters who lack an in-house execution team.

Delay

Slipped launches; idle capital

Compliance risk

Licensing, NABH & regulatory gaps

Cost leakage

Untracked spend & procurement

No accountability

Ownership scattered

03 Why now

Four tailwinds converging in our favour

Healthcare build-out

India's hospital and diagnostics capacity is expanding fast, especially in Tier-2/3 cities.

Digitisation mandate

Operators are moving off spreadsheets; NABH, GST and digital compliance push structured systems.

SaaS adoption

Indian SMBs and operators now buy vertical SaaS; cloud and mobile are default.

Formalisation

Udyam, GST and single-window reforms reward organised, auditable execution.

The window: whoever encodes real Indian hospital-and-operator execution into software first builds a compounding data and workflow advantage.



Market opportunity

A large, digitising, under-served execution market

TAM Healthcare + asset-heavy execution & the software to run it, across India

SAM Hospital execution & operator management software for Tier-1/2/3 India

SOM Doctor-led hospital projects & adjacent operators we serve first

India's healthcare market is projected into the hundreds of billions of dollars by 2030, alongside a rapidly growing vertical-SaaS market — the intersection is our arena.

Illustrative and forward-looking. Based on internal assumptions and public industry estimates; not a representation of current results or a guarantee of future performance.



05 Nine verticals, one platform

Healthcare is the flagship; the EMS runs underneath them all

Hospital Construction

Feasibility, NABH, utilities, execution

Hospital Consultancy

Licensing, SOPs, equipment, HIS

HR & Workforce

Recruitment, staffing, retention

Logistics

Fleet, bulk transport, supply chain

Mining

Site operations, compliance, supply

Import & Export

Sourcing, customs, export

E-Commerce

Store setup, marketplace, growth

PR

Positioning, media, outreach

Arbitrage

Regulatory & deal structuring

Each vertical is a wedge and a data source; the platform compounds as coverage grows.



06 The solution

One technology-enabled execution framework, powered by the EMS

Strategy to on-ground delivery, run on a single platform — doctor-led insight keeps healthcare projects compliant and clinically sound.

1

Plan

Feasibility, clinical & regulatory needs, system design, approvals, execution planning.

2

Execute

Construction, fit-out, equipment, workforce, vendors, logistics, launch coordination.

3

Operate & improve

Structured workflows, documentation, billing events, performance oversight.



07 Product architecture

A modular, role-based operating system — six live modules, eight more planned

Central Command Center

Unified control, roles, permissions, portal access, system health

Transportation & Logistics

Trips, expenses, routes, commodities, transporters, agents, trucks, settlements

Interiors

Leads, clients, projects, designs, materials, site updates, approvals, billing, closure

Digital Marketing & Services

Lead-to-billing delivery, white-label partners, client portals

Legal

Drafting, KYC, signing evidence, secure archive

Central Accounts

Journals, receivables, payables, treasury, reporting

Planned: Construction · Hospital Projects · Hospital Consultancy · Imports & Exports · Trading · HR & PR · Arbitrage · E-Commerce



Inside the platform

Depth per module — real operational software, not dashboards

Transportation & Logistics

- Dispatch KPIs: in-transit, draft trips, client outstanding, gross margin
- Master data: routes, commodities, clients, transporters, agents, trucks
- Finance action centre: receivables, payables, rate contracts, net cash

Interiors

- Lead-to-closure: leads, clients, projects, designs, materials
- Delivery: site updates, approvals, team & workforce assignment
- Billing, project closure and reporting — a live project already in-system

09 The product is live

Deployed, role-secured, with real project data — not a mock-up

Central Command Center

Super-Admin roles, system-health monitoring, three active business modules

Live project

Interiors project PRJ-00009 running through delivery and billing workflows

Finance & legal

Central Accounts and Legal modules for treasury, KYC and signing evidence

See the accompanying *EMS — Product Evidence* document for live screenshots of the running system.



10 Technology & architecture

Modern, mobile-first, built to multi-tenant

Web / PWA stack

Installable app, offline-aware, cross-device access

Modular divisions

Reusable module pattern; add a vertical without a rebuild

Roles & portals

Granular permissions; external client / transporter / interiors portals

System of record

Unified, auditable data across dispatch, projects, billing and finance

Cloud data layer

Managed, scalable backend designed for multi-tenant SaaS

Security by design

Role isolation, signing evidence, secure document archive

11 Innovation & defensibility

What is proprietary

- Original unified data model across 8+ workstreams
- Doctor-led clinical/NABH/licensing logic encoded in software
- Division framework enabling horizontal reuse across sectors

Why it compounds

- Every project and tenant enriches workflows and data
- Strategy + delivery + software under one roof — hard to copy
- Internal-first hardening yields a clean, proven SaaS core

Scalability & SaaS roadmap



Phase 1 — Internal

EMS runs our own multi-sector projects; workflows validated in production.



Phase 2 — Harden

Standardise modules, strengthen controls, prepare multi-tenant SaaS.



Phase 3 — Commercialise

License modules to hospitals and operators on subscription.

Horizontal + modular: each new vertical and tenant reuses the same core, so marginal cost of scale falls as coverage grows.

Business model

Earn from execution today; compound with recurring software

Project & implementation fees

Planning, setup and execution across hospital construction, consultancy, logistics, mining, trade.

ACTIVE PATHWAY

Operational & logistics services

Recurring fees for operations, fleet, workforce and management via the EMS.

ACTIVE PATHWAY

SaaS subscription & licensing

Per-module subscriptions for hospitals and operators as modules harden.

FUTURE · HIGH MARGIN

Services fund the build; SaaS turns operating know-how into high-margin, recurring, defensible revenue.

14 Unit economics & flywheel

Illustrative model — services-led entry, software-led expansion

Land with services

Execution engagements fund delivery teams and de-risk each account.

Expand to software

Convert operators to EMS subscriptions; low marginal cost per tenant.

Retain & grow

Modules + data lock-in drive retention and per-account expansion.

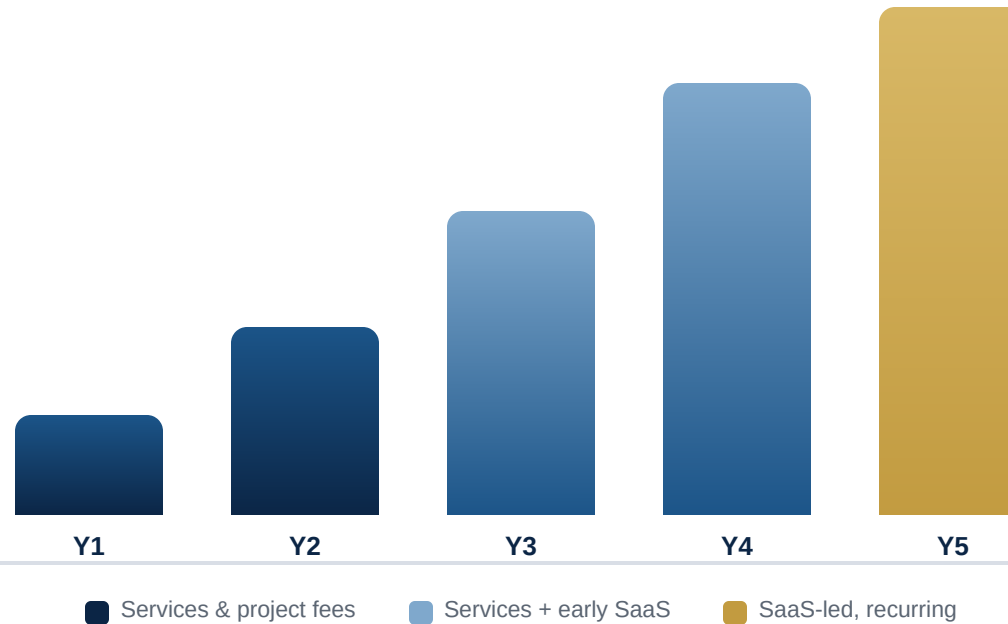
Leverage

Shared core across tenants improves gross margin as scale grows.

Illustrative and forward-looking. Based on internal assumptions and public industry estimates; not a representation of current results or a guarantee of future performance.

15 Revenue trajectory

Illustrative shift from services to recurring SaaS



The strategic goal: grow recurring, high-margin SaaS as a share of revenue year over year.

Illustrative and forward-looking. Based on internal assumptions and public industry estimates; not a representation of current results or a guarantee of future performance.

16 Go-to-market

Land with execution, expand with software

Now

Run own projects on the EMS; capture workflows; land healthcare reference deployments.

6-12 months

Harden Interiors, Transportation, Finance; onboard first external hospital pilots.

12-24 months

Multi-tenant SaaS; expand active verticals; build a partner channel.

Beachhead: doctor-led hospital execution where trust and compliance matter most — then extend outward.



17 Traction & milestones

9

Verticals

6

Live EMS modules

10

Team

1

Live in-system project

Product

EMS built and deployed internally within months of incorporation — a live, role-secured platform with real project data.

Operations

Public multi-sector offering and leadership; structured healthcare, logistics and interiors workflows in the system.



18 Competitive landscape

We combine what others do only in part

Capability	Consultants	Point vendors	Generic ERP	Varada Nexus
Doctor-led domain depth	No	No	Partial	Yes
End-to-end execution + delivery	No	Partial	No	Yes
Proprietary horizontal platform	No	No	Partial	Yes
Multi-vertical, India-built	No	No	Partial	Yes
Path to recurring SaaS	No	No	Partial	Yes

19 Alignment with DPIIT recognition criteria

Innovation in product / process / service

Proprietary EMS (product) and structured digital execution (process), not a standard commercial offering.

Scalable, high employment/wealth potential

Horizontal modular platform + recurring SaaS; on-ground teams across nine verticals.

Not split-up / reconstruction

Freshly incorporated Pvt Ltd (Feb 2025), original entity and objects.

Within age & turnover limits

Under 10 years; pre-revenue — well within recognition thresholds.

Alignment with national priorities

Startup India

Innovative, scalable venture creating employment and wealth

Digital India

Digitising paper-and-spreadsheet execution into software

Make in India

Home-grown enterprise software, built and run in India

Healthcare access

Faster, compliant hospital infrastructure in Tier-2/3 regions

21 Team & governance

Doctor-led, cross-functional, well-governed

Althi Prudhvi — Founder & Director

Medical background; execution-first philosophy — infrastructure and operations shape outcomes

Althi Suresh — Director

Co-founder; operations and corporate governance

Execution & operations

On-ground teams across healthcare, logistics and interiors

Technology

In-house build and operation of the EMS platform

Governance

Pvt Ltd, board of two directors, GST & Udyam registered

Hiring plan

Scaling domain, delivery and engineering with each vertical



The vision

From executing projects to powering an industry

Today we run hospital and asset-heavy projects on our own software. Tomorrow, that software becomes the default operating system operators across India use to plan, build, run and account for their businesses — compliant, auditable and efficient by design.

One platform. Every vertical. Built in India, for how India actually executes.



Built to scale. Grounded in execution.

Varada Nexus Private Limited